

# Tesamorelin

GHRH Analogue | FDA-Approved (Egrifta) | 5mg & 10mg x 10 Vials

Clinically Proven Visceral Fat Reduction | Once Daily Fasted

Purity: 99.8% | Endotoxin-Free | Research Use Only

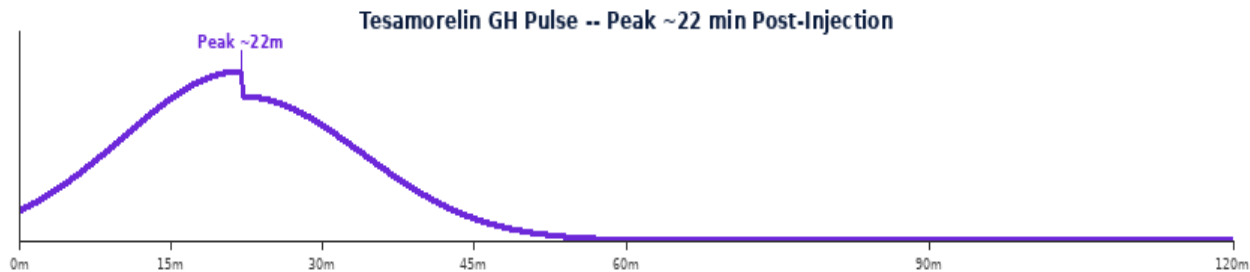
Tesamorelin is a stabilised GHRH analogue and the only FDA-approved GHRH peptide (as Egrifta for HIV-associated lipodystrophy). It stimulates physiological GH release and is the most clinically validated GHRH for visceral fat reduction. Must be injected fasted once daily.

**FASTED REQUIRED:** Inject at least 1-2 hours after last meal. Insulin blunts GH release. Wait 30 min before eating post-injection.



| Parameter      | Specification                                                    |
|----------------|------------------------------------------------------------------|
| Half-Life      | ~26-38 minutes -- stimulates natural GH pulse                    |
| Vial Sizes     | 5mg x 10   10mg x 10   Purity 99.8%                              |
| Reconstitution | 2 mL BAC Water per vial: 5mg->2.5mg/mL   10mg->5mg/mL            |
| Standard Dose  | 1,000-2,000 mcg (1-2 mg) once daily SubQ                         |
| Frequency      | Once daily -- always fasted, morning or bedtime                  |
| Key Benefit    | Clinically proven visceral fat reduction; only FDA-approved GHRH |
| Cycle          | 12-26 weeks; can run longer with monitoring                      |
| Storage        | 2-8 deg C unreconstituted   2 mL BAC Water   use within 28 days  |

MECHANISM OF ACTION & RESEARCH BENEFITS



| Pathway   | Mechanism                                                                                                                         | Benefit                                       |
|-----------|-----------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------|
| GHRHR     | Binds GHRH receptors on pituitary somatotrophs; activates Gs/cAMP/PKA; drives GH synthesis and release                            | Physiological GH pulse once daily             |
| Lipolysis | GH-mediated HSL activation; preferential reduction of visceral/abdominal adipose tissue; clinically measured by CT scan in trials | Proven visceral fat reduction; trunk fat loss |
| IGF-1     | Daily GH pulses progressively raise IGF-1; lean mass preservation; recovery enhancement over cycle                                | Body composition improvement; anti-aging      |

| Benefit        | Context                                                                                                                   |
|----------------|---------------------------------------------------------------------------------------------------------------------------|
| FDA Validation | Tesamorelin (Egrifta) is FDA-approved and has the strongest clinical evidence base of any GHRH peptide for fat reduction. |
| Visceral Fat   | CT-scan confirmed reduction in visceral adipose tissue in clinical trials -- superior evidence to other GHRH analogues.   |
| GHS Synergy    | Stacking with Ipamorelin amplifies GH pulse 3-5x. Tesamorelin + Ipamorelin is a premium body recomposition stack.         |

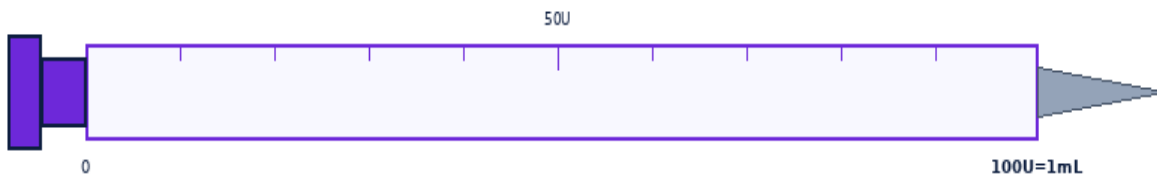
## RECONSTITUTION & DOSE CALCULATION

All vials use 1 mL BAC Water. Room temperature. Swirl gently -- never shake.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

| Step | Action           | Detail                                                                                       |
|------|------------------|----------------------------------------------------------------------------------------------|
| 1    | Gather           | Vial, BAC Water, insulin syringe (29-31G), swabs, sharps.                                    |
| 2    | Clean stoppers   | Wipe both stoppers. Air-dry 10-15 sec.                                                       |
| 3    | Draw 1mL BAC     | Exactly 1 mL. Expel air bubbles.                                                             |
| 4    | Inject into vial | Aim at GLASS WALL. Slowly. Never onto powder.                                                |
| 5    | Swirl & label    | Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days. |



1mL Insulin Syringe | 29-31G | SubQ

**Formula:** Vol(mL)=Dose(mcg)/Conc(mcg/mL) | **Units:** Vol x 100

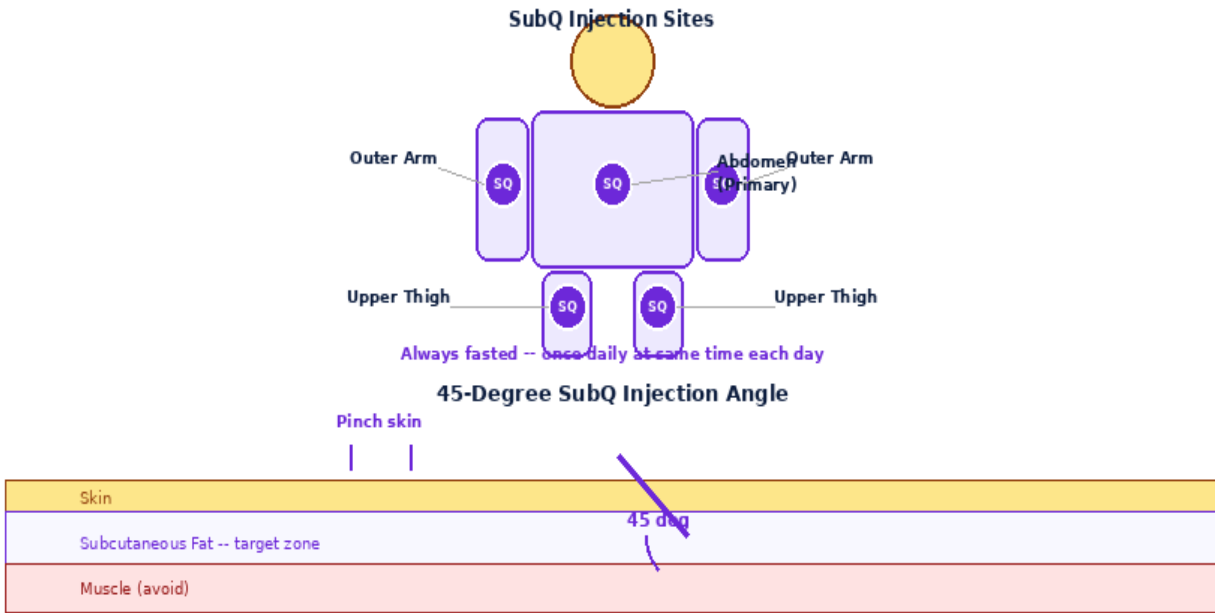
**5mg x 10 -- 2mL BAC -> 2.5mg/mL (2,500mcg/mL)**

| Dose(mcg)  | 500mcg | 750mcg | 1000mcg | 1500mcg | 2000mcg |
|------------|--------|--------|---------|---------|---------|
| Vol(mL)    | 0.2000 | 0.3000 | 0.4000  | 0.6000  | 0.8000  |
| Units      | 20.0U  | 30.0U  | 40.0U   | 60.0U   | 80.0U   |
| Doses/Vial | 5      | 3      | 2       | 1       | 1       |

**10mg x 10 -- 2mL BAC -> 5.0mg/mL (5,000mcg/mL)**

| Dose(mcg)  | 500mcg | 750mcg | 1000mcg | 1500mcg | 2000mcg |
|------------|--------|--------|---------|---------|---------|
| Vol(mL)    | 0.1000 | 0.1500 | 0.2000  | 0.3000  | 0.4000  |
| Units      | 10.0U  | 15.0U  | 20.0U   | 30.0U   | 40.0U   |
| Doses/Vial | 10     | 6      | 5       | 3       | 2       |

## INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



8-Point Rotation Map -- Rotate Every Injection



| Step | Action         | Detail                                                 |
|------|----------------|--------------------------------------------------------|
| 1    | Confirm fasted | 1-2 hr after last meal. Mandatory.                     |
| 2    | Draw dose      | Draw 0.4-0.8mL (1,000-2,000 mcg) per dose table.       |
| 3    | Inject SubQ    | Abdomen preferred. Pinch 2-3cm. 45 deg. Inject slowly. |
| 4    | Wait & rotate  | Wait 30 min before eating. Rotate 8-point map.         |

| Protocol   | Dose                     | Frequency            | Duration | Notes                                        |
|------------|--------------------------|----------------------|----------|----------------------------------------------|
| Standard   | 1,000 mcg                | 1x daily fasted      | 12-26 wk | Morning or bedtime fasted.                   |
| High       | 2,000 mcg                | 1x daily fasted      | 12-26 wk | Max FDA dose. Monitor IGF-1.                 |
| +IPA Stack | 1,000 mcg +200mcg<br>Ipa | Both daily<br>fasted | 12 wk    | Inject simultaneously. Best recomp protocol. |

| Side Effect                  | Likelihood      | Management                             |
|------------------------------|-----------------|----------------------------------------|
| Injection site reaction      | Common          | Rotate sites. Use fresh needle.        |
| Water retention              | Occasional      | GH-mediated. Reduce dose.              |
| Glucose changes              | Occasional      | Monitor fasting glucose at high doses. |
| Do not use in cancer history | Contraindicated | GH/IGF-1 elevation risk.               |

**Contraindications: Cancer history, pregnancy, under 18.**

|                                                                                        |
|----------------------------------------------------------------------------------------|
| 1. Inject fasted (1-2hr after last meal). Insulin blunts GH release.                   |
| 2. 2 mL BAC Water per vial: 5mg->2.5mg/mL   10mg->5mg/mL.                              |
| 3. Once daily injection -- morning fasted or pre-bed are optimal.                      |
| 4. Stack with Ipamorelin for amplified GH pulse and superior body composition results. |
| 5. Do not use in any cancer history -- GH/IGF-1 stimulation is contraindicated.        |
| 6. Rotate injection sites. Store 2-8 deg C. Use within 28 days.                        |
| 7. Cycle 12-26 weeks. Monitor IGF-1 levels periodically on long cycles.                |

---

DISCLAIMER: SaiyanMed research use only. Not for human therapeutic use. Consult a qualified professional. SaiyanMed assumes no liability.

**SaiyanMed | [saiyanmed.com](https://saiyanmed.com) | Research Use Only**